

1. Administrative & Study Identification

Full Study Title: A Phase II, Open-Label, Multicenter, Platform Study Evaluating the Efficacy and Safety of Biomarker-Driven Therapies in Patients With Persistent or Recurrent Rare Epithelial Ovarian Tumors **Short Title/Acronym:** BOUQUET Trial
Protocol Number: W042178 (Associated IDs: GOG-3051, ENGOT-GYN2)
NCT Number: NCT04931342 **Posting ID:** 897395762950279385 **Trial Status:** Active, not recruiting (ACTIVE_NOT_RECRUITING) **Sponsor/ Company Name:** Hoffmann-La Roche / Genentech **Investigational Product:** Multiple biomarker-driven treatments (e.g., Ipatasertib + Paclitaxel, Cobimetinib, Trastuzumab emtansine, Atezolizumab + Bevacizumab, Giredestrant + Abemaciclib +/- LHRH, Inavolisib + Palbociclib +/- Letrozole, Cyclophosphamide [Preferred Name: cyclophosphamide; ATC Code: L01AA01; Trade Names: Cytosan, Endoxana]) **Phase of Development:** Phase II **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy and safety of multiple biomarker-selected treatments in patients with persistent or recurrent rare epithelial ovarian, fallopian tube, or primary peritoneal tumors. **Secondary Objectives:** To evaluate Progression-Free Survival (PFS), Overall Survival (OS), Duration of Response (DoR), Disease Control Rate (DCR), and specific safety and tolerability profiles of each experimental combination regimen.

2.2 Background & Rationale To address the significant unmet medical need in patients with rare epithelial ovarian cancers (such as low-grade serous, clear cell, mucinous, or carcinosarcoma) who have progressed on standard platinum-based therapies. Because rare ovarian tumors have distinct genetic mutations compared to common high-grade serous ovarian cancer, this platform study evaluates whether matching specific targeted therapies to individual tumor biomarkers (e.g., PIK3CA, AKT1, BRAF, KRAS, ERBB2, ER+) improves clinical outcomes.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional (Platform Study)
Control Type: Single-arm (per experimental biomarker cohort)
Blinding: Open-label **Randomization:** Non-randomized (Assignment based on biomarker screening)

3.2 Planned Enrollment & Duration Target Enrollment:

Approximately 176 participants **Number of Sites:** Over 50 international institutes (coordinated via ENGOT, GINECO, and others) **Estimated Study Start:** 2021 **Trial Status:** Active but not currently recruiting **Estimated Primary Completion:** TBD (Event-driven based on disease progression criteria)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Persistent or recurrent EOC meeting specific criteria: Histologically confirmed non-high-grade serous, non-high-grade endometrioid epithelial ovarian, fallopian tube, or primary peritoneal cancer (including low-grade serous, clear cell, mucinous, carcinosarcoma, etc.). 2. Measurable disease (at least one target lesion) according to RECIST v1.1. 3. Previous treatment with one to four lines of therapy, at least one of which was platinum-based. 4. Platinum-resistant disease (progression during or within 6 months of last platinum therapy, excluding primary platinum-refractory disease). 5. Submission of a representative tumor specimen suitable for next-generation sequencing (NGS) testing and estrogen receptor immunohistochemistry (ER IHC). 6. ECOG Performance Status of 0 or 1.

4.2 Exclusion Criteria 1. Pregnant or breastfeeding, or intending to become pregnant or breastfeed during the study. 2. Primary platinum-refractory disease, defined as progression during or within 4 weeks after the last dose of the first-line platinum treatment. 3. Histologic diagnosis of high-grade serous or high-grade endometrioid ovarian cancer, current diagnosis of solely borderline epithelial ovarian tumor, or non-epithelial ovarian tumors. 4. Synchronous primary endometrial cancer or unpermitted prior history of primary endometrial cancer. 5. Symptomatic, untreated, or actively progressing CNS metastases. 6. Treatment with chemotherapy, radiotherapy, antibody therapy, or investigational therapy within 28 days prior to initiation of study treatment.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Cycle frequency depends on the specific biomarker-driven arm (either 21-day or 28-day cycles). Regimens include combinations such as daily pills or intravenous infusions, utilizing agents like targeted inhibitors or cyclophosphamide. **Route of Administration:** Oral (tablets) and Intravenous (infusions), depending on the assigned experimental arm. **Duration of Treatment:** Until unacceptable toxicity or disease progression per RECIST v1.1.

5.2 Concomitant & Prohibited Therapy Permitted: Routine supportive care for symptomatic management. **Prohibited:** Other

concurrent investigational therapies or anticancer therapies.
Note: For premenopausal or perimenopausal women in the ER+ (hormone-driven) arms, LHRH agonists (e.g., goserelin or leuprolide) are strictly required beginning at least 2 weeks prior to initiation of study treatment.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Tumor Biopsy Submission, NGS/ER IHC Biomarker Screening, Baseline Scans
Baseline	Day 1 (Cycle 1)	Cohort Assignment, First Dose of Targeted Therapy, Safety Labs
Interim	Per Cycle (21/28d)	Safety Labs, AE Assessment, Medication Dispensing/Infusions (e.g., Day 1, 8, 15)
End of Study	Off-treatment	Treatment Discontinuation due to Disease Progression (RECIST v1.1) or Toxicity, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Confirmed Objective Response Rate (ORR), defined as the proportion of participants with a confirmed complete response (CR) or partial response (PR) (demonstrated on two consecutive occasions ≥ 4 weeks apart).
Measurement Method: Radiologic tumor assessment using RECIST v1.1 criteria and standard adverse event scaling (CTCAE).

7.2 Secondary & Exploratory Endpoints 1. Duration of Response (DoR) 2. Disease Control Rate (DCR) 3. Progression-Free Survival (PFS) 4. Overall Survival (OS)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard assessment of known unwanted effects (e.g., gastrointestinal issues, neutropenia, rash, infusion-related reactions) monitored continuously throughout treatment cycles. **Serious Adverse Events (SAEs):** Protocol-mandated reporting timelines to the Sponsor and local ethics committees. **Data Monitoring Committee (DMC):** Internal/External monitoring committees to oversee the safety of the multi-arm platform architecture.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy bounds per cohort; Safety Set for all participants receiving at least one dose of the assigned drug. **Sample Size Justification:** A platform trial designed to enroll approximately 176 participants

into biomarker-matched cohorts to evaluate signal-finding efficacy (Phase II estimates). **Handling of Missing Data:** Standard oncologic censoring rules applied for time-to-event analyses (PFS, OS).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring visits to ensure integrity of data, protocol compliance, and central pathology review adherence. **Audit Trail:** Standard electronic Case Report Form (eCRF) tracking and data clarification processes.

11. Study Identifiers & Governance

Primary ID: W042178 **Registry Number:** NCT04931342 **Posting ID:** 897395762950279385 **Sponsor/Lead Organization:** Hoffmann-La Roche / Genentech **Study Title:** BOUQUET Trial **Official Regulatory Title:** A Phase II, Open-Label, Multicenter, Platform Study Evaluating the Efficacy and Safety of Biomarker-Driven Therapies in Patients With Persistent or Recurrent Rare Epithelial Ovarian Tumors

12. Clinical Framework (Oncology Specific)

Disease Area: Ovarian Cancer **Preferred UMLS Name:** Recurrent Epithelial Ovarian Neoplasm **Primary Condition:** Rare Epithelial Ovarian, Fallopian Tube, or Primary Peritoneal Tumors **Diagnosis Threshold:** Relapsed/Refractory non-high-grade serous, non-high-grade endometrioid histology **Medical Methodology:** Next-Generation Sequencing (NGS) and ER Immunohistochemistry (IHC) Biomarker Profiling **Optimization Target:** Objective Radiologic Tumor Response (ORR) per RECIST v1.1

13. Study Procedures & Methodology

Management Pathway: Centralized biomarker profiling → Assignment to matched targeted therapy arm → Assessment of progression **Education Protocol (HCP):** Site Initiation Visits (SIV) and platform protocol training for 53+ global sites **Education Protocol (Patient):** Informed consent detailing potential genomic matches and associated targeted therapies **Quality Audit Mechanism:** Centralized pathology review to confirm rare histology and specific biomarker status prior to enrollment

14. Observation & Follow-Up Schedule

Total Duration: Event-driven (until disease progression or unacceptable toxicity) **Onsite Visit Cadence:** Depending on arm; primarily Day 1 (and Days 8/15 for some IV treatments) of 21-day or 28-day cycles **Remote Monitoring Frequency:** Routine adverse event and survival tracking **Checklist Review Interval:** Radiologic tumor assessments every 8-12 weeks per standard oncologic intervals

15. Sign-off & Approval

Role	Name	Signature	Date						
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]						
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]						
Lead Statistician	[Name]	_____	[DD-MMM-YYYY]						